

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction
Wegovy (semaglutide) Tablets
for weight management

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must be competent in weight management consultation, BMI calculation and the recognition of red flags requiring referral • All users must previously have used PGD's to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005

Initial Assessment

Before commencing treatment with weight management therapies, the patient must be assessed by a registered healthcare professional. The initial assessment should be face to face. The assessment should include, but may not be limited to:

- Causes of weight gain. Refer the patient to their GP if prescribed medication is causing weight gain.
- The patient's lifestyle, diet and level of exercise.
- Has the patient tried to lose weight in the past? Discuss what has and has not worked.
- Other factors that may be causing weight gain, including mental health, environmental and psychological factors. Consider signposting to another health professional, for example a mental health team.
- Other disease states that may predispose to weight gain. Consider referring to the GP for further advice.
- The patient's expectations of weight loss, and whether they are realistic for their height and body frame.
- Calculate BMI and determine the ideal weight. Set a realistic target weight and agree review intervals.

Clinical condition or situation to which this PGD applies

PGD Indication	Wegovy (semaglutide) tablets are a UK-licensed oral GLP-1 receptor agonist indicated as an adjunct to a reduced-calorie diet and increased physical activity for weight management, including weight loss and weight maintenance, in adults with an initial Body Mass Index (BMI) of 30 kg/m ² or above (obesity), or 27 kg/m ² to below 30 kg/m ² (overweight)
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	in the presence of at least one weight-related comorbidity, for example hypertension, dyslipidaemia, obstructive sleep apnoea, cardiovascular disease or type 2 diabetes. This medicine is subject to additional monitoring (black triangle); report all suspected adverse reactions via the MHRA Yellow Card scheme.
Inclusion criteria	• Adults aged 18 to 85 years (inclusive). • BMI 30 kg/m² or above, or BMI 27 kg/m² or above with at least one weight-related comorbidity (for example hypertension, type 2 diabetes mellitus, dyslipidaemia, obstructive sleep apnoea, established cardiovascular disease). • Patient is willing to follow a reduced-calorie diet and increase physical activity in line with the agreed lifestyle plan. • Initial assessment completed and documented (see the Initial Assessment section above). • Informed consent obtained, including off-label considerations where applicable, the side-effect profile and treatment expectations. • No exclusion criteria present. • Patient is able to follow the empty-stomach administration requirements: at least an 8 hour fast before the dose, and a 30 minute wait afterwards before food, drink or other oral medicines.
Exclusion criteria	• Adults under 18 years of age. • Adults aged over 85 years (upper age limit under this PGD; refer to a specialist if treatment is being considered). • Pregnancy, breastfeeding, or planning pregnancy. Effective contraception is required throughout treatment. Semaglutide must be discontinued at least 2 months before a planned pregnancy because of its long half-life, and stopped immediately if pregnancy occurs or is suspected (SPC section 4.6). Tirzepatide is not covered by this PGD. • Known hypersensitivity to semaglutide or to any of the excipients. • Personal or family history of medullary thyroid carcinoma (MTC) or Multiple Endocrine Neoplasia syndrome type 2 (MEN 2). • History of pancreatitis, acute or chronic. • Severe gastrointestinal disease, including gastroparesis or severe persistent gastrointestinal disorder. • Current cholelithiasis (gallstones) or cholecystitis, or cholecystectomy within the last 3 months. • Obesity caused by an endocrinological disorder. If the patient was already overweight prior to that diagnosis, this exclusion may not apply. • Concurrent use of any other GLP-1 receptor agonist or insulin secretagogue used for weight management. • Type 1 diabetes mellitus. • Diabetic retinopathy. Treatment may worsen retinopathy; defer or refer to a specialist. • Patients on insulin or sulphonylureas whose GP is unwilling to monitor and adjust their hypoglycaemic regimen. • Severe renal impairment (eGFR below 30 mL/min/1.73 m²) or end-stage renal disease. • Severe hepatic impairment.

	• Known diagnosis of heart failure with reduced ejection fraction below 40%. • Active eating disorder (anorexia nervosa, bulimia, or binge-eating disorder under specialist care). • BMI below the PGD inclusion threshold. • Patients who, in the clinical judgement of the healthcare professional, are not suitable for the medication.
Cautions	• Counsel on gradual dose escalation and gastrointestinal side effects; consider delaying titration or reducing to the previous dose if significant symptoms occur. • History of suicidal ideation, or active severe mental illness. Ensure appropriate psychiatric oversight is in place, monitor mood at review, and refer if there is any concern. Do not supply where oversight is absent and concern exists. • Non-arteritic anterior ischaemic optic neuropathy (NAION) has been reported with semaglutide. Advise patients to seek urgent review for any sudden loss of vision, and discontinue if NAION is confirmed. • Mild to moderate renal impairment: monitor for dehydration secondary to gastrointestinal side effects. • Concomitant oral medications: delayed gastric emptying may reduce the absorption of oral medicines, especially those with a narrow therapeutic index. Counsel accordingly. • For individuals who take HRT, due to the lack of data regarding absorption, non-oral products (for example patch, gel, or levonorgestrel intrauterine device) may be considered. • Semaglutide delays gastric emptying and thereby has the potential to impact the rate of absorption of concomitantly administered oral medicinal products, especially those with a narrow therapeutic index. Upon initiation of semaglutide treatment in patients on warfarin, frequent monitoring of INR is recommended. Decreased INR has been reported with acenocoumarol, so the same applies to other coumarins. • Levothyroxine. Oral semaglutide increases levothyroxine exposure by about a third (AUC increased 33%). Monitor thyroid function when the two are taken together, and make sure the patient keeps the 30 minute separation, which matters more here than with most co-medicines. Cases of pulmonary aspiration have been reported in patients receiving GLP-1 receptor agonists undergoing general anaesthesia or deep sedation for surgery. The increased risk of residual gastric content due to delayed gastric emptying should therefore be considered before procedures under general anaesthesia or deep sedation. • Cases of tachycardia have been reported. In patients with a pre-existing increased heart rate, use with caution and seek specialist advice before use. If a patient experiences a clinically relevant sustained increase in resting heart rate, treatment should be discontinued and advice sought. • Counsel on the signs and symptoms of pancreatitis; discontinue and refer urgently if suspected. • Acute pancreatitis: the MHRA strengthened the class warning for GLP-1 receptor agonists on 29 January 2026 after reports of necrotising and fatal cases. Make the red flag explicit to the patient: severe, persistent abdominal pain, often radiating to the back, means stop the medicine and seek urgent medical attention

	the same day. • The 25 mg maintenance tablet contains 23 mg of sodium. The lower strengths are essentially sodium free. This is rarely material but is worth knowing for a patient on a sodium restricted diet. • Upper age limit of 85 years under this PGD. Note this is a Get Real Health position, not a licence restriction: the SPC sets no upper age limit and simply records that experience above 85 is limited. • Sulfonylurea or insulin co-use in patients with comorbid type 2 diabetes carries a hypoglycaemia risk. Refer to the prescribing GP or specialist before starting. • Risk of dehydration. Counsel on adequate fluid intake during gastrointestinal side effects. • Reassess the benefit-risk balance if there is no clinically meaningful weight loss, typically less than 5%, after 6 months on the maintenance dose.
Actions if patient is excluded or declines treatment	• Discuss the reason for exclusion with the patient and ensure they understand. • Advise on alternative treatment options and how these can be accessed, for example the GP, a specialist weight management service, or lifestyle programmes. • If the exclusion relates to an undiagnosed or unmanaged comorbidity, recommend GP review. • Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Wegovy (semaglutide) tablets: 1.5 mg, 4 mg, 9 mg and 25 mg. Black triangle medicine subject to additional monitoring. Record the product name and batch number for traceability.
Legal category	POM
Storage	Store in the original package below 30°C in order to protect from moisture. Keep the container tightly closed.
Route/method of administration	• Oral, once daily, on an empty stomach after a fast of at least 8 hours, with no more than half a glass of water (about 120 mL). • Wait at least 30 minutes before food, drink or other oral medicines. • Swallow the tablet whole. Do not split, crush or chew. • Only one tablet per day. Never combine tablets to approximate a higher dose.
Dose and frequency	• Start at 1.5 mg once daily for one month. Escalate monthly through 4 mg and 9 mg to the maintenance dose of 25 mg once daily, with a minimum of one month at each step. If needed, the dose can be maintained at the previous level. • Maximum dose 25 mg once daily. • Missed dose: skip it and take the next dose the following day. • If a dose is missed, skip it and take the next dose the following day. Never take two tablets in a day to make up a missed dose (SPC section 4.2). Where several consecutive doses have been missed, use clinical judgement about restarting at a lower step and re-escalating; that is a practice decision rather than a licensed instruction. • In case of significant gastrointestinal symptoms during titration,

	consider delaying a dose increase or lowering to the previous dose until symptoms have improved. • If the patient wants to recommence treatment, the dose should be titrated again starting with the lowest dose. The BMI inclusion criteria for initiation must be applied if more than 2 months have passed since discontinuing treatment. • Switching from semaglutide 2.4 mg injection: 25 mg once daily, starting one week after the last injection.
Quantity to be administered and/or supplied	• One calendar pack of 30 tablets at the appropriate strength, giving one month of treatment. • Supply one month of treatment per patient appointment. • This PGD does not allow additional medication to be supplied to enable patients to stock up.
Monitoring and review	• Reassess clinical benefit, tolerability and target weight at each visit. • If the patient has not lost at least 5% of their initial body weight after 6 months on the maximum tolerated dose, a decision is required on whether to continue treatment. • When the patient reaches their target weight, discuss whether to continue treatment to maintain it. • Confirm at each visit that the empty-stomach administration requirements are being followed, as absorption depends on them.
Adverse effects	Very common: nausea, diarrhoea, vomiting, constipation and abdominal pain. Common: dyspepsia, eructation, flatulence, gastro-oesophageal reflux, gallstones, headache, fatigue, dizziness, hypoglycaemia when used with a sulfonylurea or insulin, hair loss and tachycardia. Uncommon or rare: acute pancreatitis, cholecystitis, hypersensitivity reactions including anaphylaxis, acute kidney injury usually secondary to dehydration, and non-arteritic anterior ischaemic optic neuropathy. Refer to the current SmPC for full safety information.
Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP with whom the individual is registered • name of healthcare practitioner • name and brand of medication, and batch number • date of supply, dose, form, route and quantity supplied • height, weight and BMI at this visit, and the target weight agreed • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken • that the medicine was supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. Adults aged 18 and over: keep records for 8 years.

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medicine, together with written lifestyle, diet and physical activity advice and the agreed target weight.
Follow-up advice to be given to	Explain the expected pattern of weight loss and that the medicine works

patient or carer	alongside diet and activity, not instead of them. Counsel on gastrointestinal side effects and how to manage them, on adequate fluid intake, and on the warning symptoms that need urgent attention: severe abdominal pain, persistent vomiting with dehydration, jaundice, sudden visual loss, or a sustained rise in resting heart rate. Advise when to return for review and that treatment will be reassessed if less than 5% of body weight has been lost after 6 months on the maintenance dose.
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Key references

• Summary of Product Characteristics for the product supplied, current version • NICE guidance on obesity management and on the relevant medicine • MHRA Drug Safety Updates relevant to GLP-1 receptor agonists • British National Formulary (BNF) • NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions
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Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates	

Name of healthcare professional	Job title	Registration number	Signature

Valid from date	Expiry date
23/8/26	30/9/27

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or vaccine.

This section must be signed by the superintendent or clinical lead responsible for this service.

To be completed by the adopting pharmacy: the first block is signed by that pharmacy's superintendent or clinical lead, and the second by its professional group signatory. Get Real Health signs the authorship block below, not these.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional	Job title & name of	Signature	Date

group signatory	organisation		

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		23/8/26
Chris Pilkington	Pharmacist GPhC: 2046322		23/8/26

Change history

Version number	Change details	Date
001	Development and issue of new PGD	10th July 2026
002	PPH clinical review (J. Wilkins): 19 amendments including dosing, administration and monitoring detail.	23rd July 2026
003	PPH clinical review (J. Wilkins with S. Passmore). Inclusions, exclusions and cautions aligned with the Wegovy injection PGD, with the upper age limit kept at 85 years for the tablet against 75 for the injection. Adjusted BMI thresholds for ethnic groups removed. Gallbladder disease moved from cautions to exclusions. Obesity caused by an endocrinological disorder added as an exclusion. History of suicidal ideation or active severe mental illness moved from exclusions to cautions. Cautions added covering HRT absorption, delayed gastric emptying with INR monitoring on warfarin, pulmonary aspiration under general anaesthesia or deep sedation, and tachycardia. Dosing now covers missed doses, gastrointestinal symptoms during titration, and re-titration from the lowest dose with BMI criteria reapplied after 2 months.	21st August 2026